

Our Ref: (T) // [MED-ID]

[DATE]

Clinic Letter [DATE]

[ADDRESS]

A Level

Cosham

Portsmouth

[POSTCODE]

Division C

Care Group - Child Health Paediatric Gastroenterology

Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Problem: Ileocaecal Crohn's disease (diagnosed November 2009).

Current Management:

Azathioprine 125 milligrams once daily (started February 2010, increased June 2010)

Pentasa 1 gram twice daily - taken erratically

Multivitamin and mineral supplement

Plan:

Three monthly bloods - Stanley to liaise with his General Practitioner.

Referral to [PROVIDER-NAME], Adult Gastroenterologist for follow-up.

I would be grateful if you would take on the long-term management of this young man.

He was first seen by me in November 2009 with a six month history of recurrent gut symptoms with raised inflammatory markers. He had endoscopy done here which showed a significant ileitis mild colitis (worse proximally) with features entirely consistent with Crohn's disease.

He was treated initially with Modulen IBD as a complete feed with a good impact on his symptoms flared requiring corticosteroids in 2010 requiring a second course of Modulen IBD and then corticosteroids. He had azathioprine introduced in February 2010 and has in general once that from the point of view of his Crohn's disease being better since with his last steroid course in June 2010. He was reinvestigated here in April 2010 by barium radiologist/ultrasound scan minor ulceration at the terminal ileum but no stricturing.

He has, in addition to his Crohn's disease had significant problems with his mood manifest by loss of confidence and the difficulty coming to terms with his diagnosis. He did for a period issue with eating and had input through our Child Psychiatry Unit here for some months during I reviewed him in Portsmouth on 13 August alongside [PROVIDER-NAME].

He is now 18 who is really quite well at the moment. He has intermittent gut symptoms but not particularly significant. He is managing to eat more consistently and weight was better than (height 178.4 cm, 50 th centile, weight 61.55 kg, 9 th to 25 th centile).

He has just completed his A levels and is hoping to go to Edinburgh University.

His attendance in clinic has been some erratic of late. I checked his bloods in Portsmouth. I emphasised to him that he needs to liaise with his General Practitioner direct and continue to have monthly bloods done through primary care. His TPMT was normal and he has had no problems with azathioprine tolerance.

I have said that his long-term care is best now coordinated through the adult gastroenterologist and that although at University soon would be appropriate for him to be well known to the local team. I would be grateful therefore if you would see him for assessment and take on his long-term care.

With kind regards

Yours sincerely

[PROVIDER-NAME]

Consultant Paediatric Gastroenterologist

Copy to:

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]
[ADDRESS]
[ADDRESS]
[POSTCODE]